

ADMISSION NOTE - Patient synth_04 - Acute gastroenteritis with dehydration and AKI

TIRUMALA MULTISPECIALITY HOSPITAL, HYDERABAD

PATIENT NAME: Mr. Prakash Sharma

AGE/SEX: 52/M

UHID: 240510-9832

ADMISSION DATE: 10/05/24

DIAGNOSIS:

1. Acute Gastroenteritis with Moderate Dehydration
2. Hyponatremia
3. Acute Kidney Injury (AKI) on evaluation

CHIEF COMPLAINTS:

1. Multiple loose stools x 2 days
2. Vomiting x 1 day
3. Decreased urine output x 1 day

HISTORY OF PRESENT ILLNESS:

Patient, a 52-year-old male, was in his usual state of health until 2 days ago when he developed loose, watery stools, about 15-20 episodes per day, non-mucoid, not associated with blood. This was followed by 5-6 episodes of non-bilious, non-bloody vomiting since yesterday, containing food particles. He complains of generalized weakness, dizziness on standing and significant reduction in urine volume for the past 24 hours. He tried over-the-counter medication (Lopiramide) with no relief. Due to persisting symptoms and increasing weakness, he has presented here for further management.

PAST HISTORY:

Known case of Hypertension for 5 years, on regular treatment with TAB TELMA-H (40mg/12.5mg) 1-0-0. No known history of Diabetes Mellitus, Tuberculosis, Asthma, or Thyroid disorder. No known drug allergies.

PERSONAL HISTORY:

Mixed diet. Non-smoker. Occasional alcohol consumption.

GENERAL EXAMINATION:

Patient is conscious, oriented, appears tired and dehydrated.

VITALS: BP: 90/60 mmHg, PR: 110/min, feeble, RR: 18/min, SpO2: 97% at room air, Temp: 99.0°F.

Signs of dehydration present: dry tongue, decreased skin turgor, sunken eyes. No pallor, icterus, cyanosis, clubbing or pedal edema.

SYSTEMIC EXAMINATION:

CVS: S1, S2 heard. Tachycardia present. No murmurs.

RS: Air entry bilaterally equal, no added sounds.

P/A: Soft, non-tender. Bowel sounds are hyperactive.

CNS: GCS 15/15. No focal neurological deficits.

INVESTIGATIONS:

Blood sent for CBC, RFT, S. Electrolytes. Urine sent for R/M and C/S. Reports awaited.

PLAN:

- Admit in ward.
- IV Fluids: Start with 1 pint RL stat, then alternate NS and RL.
- INJ EMESET 4mg IV SOS for vomiting.
- INJ PAN-40 40mg IV 1-0-0.
- TAB OFLOX-TZ 1-0-1.
- TAB ENTROL 1-0-1.
- Strict Intake-Output charting. Monitor vitals.

PROGRESS NOTE ? Day 1 - Patient synth_04 - Acute gastroenteritis with dehydration and AKI

TIRUMALA MULTISPECIALITY HOSPITAL, HYDERABAD

PROGRESS NOTE

PATIENT NAME: Mr. Prakash Sharma, 52/M

DATE: 11/05/24

S: Patient reports feeling slightly better. Vomiting has stopped. Frequency of loose stools has reduced to 8-10 episodes in last 24 hrs. Passed about 600ml urine since admission. Still feels weak.

O:

VITALS: BP 100/70 mmHg, PR 96/min, RR 18/min, Temp Afebrile.

General condition: Looks less dehydrated, tongue is moist. Accepting sips of oral fluids.

INVESTIGATIONS:

CBC: Hb 14.5 g/dL, TLC 11,200/cumm (Neutrophilia)

RFT (on admission): Urea 88 mg/dL, Creatinine 1.9 mg/dL

S. Electrolytes: Na⁺ 130 mEq/L, K⁺ 3.9 mEq/L

A: Acute Gastroenteritis with pre-renal AKI and Hyponatremia, showing signs of improvement with hydration.

P:

- Continue IV fluids (NS + 20Kcl) at 100ml/hr.
- Continue INJ PAN-40 1-0-0.
- Continue TAB OFLOX-TZ 1-0-1 and TAB ENTROL 1-0-1.
- INJ EMESET SOS.
- Encourage oral rehydration with ORS and soft diet as tolerated.
- Repeat RFT and electrolytes in the morning.
- Await Urine Culture report.

PROGRESS NOTE ? Day 2 - Patient synth_04 - Acute gastroenteritis with dehydration and AKI

TIRUMALA MULTISpeciality HOSPITAL, HYDERABAD

PROGRESS NOTE

PATIENT NAME: Mr. Prakash Sharma, 52/M

DATE: 12/05/24

S: Patient feels much better today. No episodes of vomiting. Passed 2 semi-formed stools since morning. Tolerating soft diet and oral fluids well. No giddiness or weakness.

O:

VITALS: BP 120/80 mmHg, PR 82/min, RR 16/min, Temp Afebrile.

General condition: Looks well-hydrated, active and comfortable. Abdomen is soft.

INVESTIGATIONS:

Repeat RFT: Urea 45 mg/dL, Creatinine 1.2 mg/dL

Repeat S. Electrolytes: Na⁺ 136 mEq/L

A: Resolving Gastroenteritis. AKI and electrolyte imbalance corrected. Patient is hemodynamically stable and fit for discharge.

P:

- IV cannula removed. Shift to oral medications completely.
- Stop home medication TAB TELMA-H for 2 more days, then restart.
- Discharge planned for today with advice.
- Continue antibiotics and supportive medications.
- Explain discharge medications and follow-up plan.

LABORATORY REPORT - Patient synth_04 - Acute gastroenteritis with dehydration and AKI

TIRUMALA MULTISPECIALITY HOSPITAL - LABORATORY REPORT

PATIENT NAME: Mr. Prakash Sharma, 52/M

UHID: 240510-9832

SAMPLE COLLECTION DATE: 10/05/24 & 12/05/24

--- COMPLETE BLOOD COUNT (10/05/24) ---

PARAMETER VALUE UNITS REFERENCE RANGE

HEMOGLOBIN 14.5 g/dL 13.0 - 17.0

TOTAL LEUCOCYTE CT. 11,200 /cumm 4,000 - 11,000

PLATELET COUNT 2.8 lakhs/cumm 1.5 - 4.5

--- RENAL FUNCTION TEST (10/05/24) ---

PARAMETER VALUE UNITS REFERENCE RANGE

BLOOD UREA 88 mg/dL 15 - 40

SERUM CREATININE 1.9 mg/dL 0.7 - 1.2

--- RENAL FUNCTION TEST (12/05/24) ---

PARAMETER VALUE UNITS REFERENCE RANGE

BLOOD UREA 45 mg/dL 15 - 40

SERUM CREATININE 1.2 mg/dL 0.7 - 1.2

--- SERUM ELECTROLYTES (10/05/24) ---

PARAMETER VALUE UNITS REFERENCE RANGE

SODIUM (Na+) 130 mEq/L 135 - 145

POTASSIUM (K+) 3.9 mEq/L 3.5 - 5.0

--- URINE ROUTINE & MICROSCOPY (10/05/24) ---

PARAMETER VALUE

COLOR Pale Yellow

APPEARANCE Slightly Turbid

PUS CELLS 2-3 /hpf

RED BLOOD CELLS Nil

--- URINE CULTURE & SENSITIVITY (10/05/24) ---

STATUS: Report Awaited.

MEDICATION RECORD - Patient synth_04 - Acute gastroenteritis with dehydration and AKI

TIRUMALA MULTISPECIALITY HOSPITAL, HYDERABAD

PATIENT NAME: Mr. Prakash Sharma, 52/M

UHID: 240510-9832

--- ADMISSION MEDICATIONS ---

1. INJ RINGER'S LACTATE - 500ml IV STAT, then as per fluid plan
2. INJ NORMAL SALINE - 500ml IV, alternating with RL
3. INJ EMESET 4mg IV SOS
4. INJ PAN-40 40mg IV 1-0-0
5. TAB OFLOX-TZ (Ofloxacin 200mg + Tinidazole 600mg) - 1-0-1
6. TAB ENTROL (Racecadotril 100mg) - 1-0-1

--- DISCHARGE MEDICATIONS ---

1. TAB AZITHRAL 500mg (Azithromycin) - 1-0-0 for 3 days
2. TAB PAN-40 40mg - 1-0-0 (before breakfast) for 5 days
3. TAB ENTROL 100mg - 1-0-1 for 3 days
4. TAB DOLO-650 (Paracetamol) - SOS for fever/body ache
5. ORS Powder - As required for hydration

DISCHARGE ADVICE - Patient synth_04 - Acute gastroenteritis with dehydration and AKI

TIRUMALA MULTISPECIALITY HOSPITAL, HYDERABAD
DISCHARGE SUMMARY

PATIENT NAME: Mr. Prakash Sharma
AGE/SEX: 52/M
UHID: 240510-9832
DATE OF ADMISSION: 10/05/24
DATE OF DISCHARGE: 12/05/24

- DIAGNOSIS:
- 1. Acute Gastroenteritis with moderate dehydration (Resolved)
 - 2. Pre-renal Azotemia (Resolved)
 - 3. Hyponatremia (Corrected)
 - 4. Hypertension

COURSE IN THE HOSPITAL:

Mr. Prakash Sharma was admitted with complaints of loose stools, vomiting and decreased urine output. Investigations revealed dehydration, hyponatremia and acute kidney injury. He was managed conservatively with IV fluids, antibiotics and other supportive care. His symptoms resolved, and his kidney function and electrolytes normalized. He is now hemodynamically stable and tolerating oral diet.

CONDITION AT DISCHARGE:

Stable, afebrile, well-hydrated. Vitals are normal. Accepting oral fluids and diet.

ADVICE ON DISCHARGE:

No	Medication Name	Dosage	Frequency	Duration
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1	TAB AZITHRAL	500mg	1-0-0	3 Days
2	TAB PAN-40	40mg	1-0-0 (Pre-BF)	5 Days
3	TAB ENTROL	100mg	1-0-1	3 Days
4	TAB DOLO-650	650mg	SOS	As needed

- Continue plenty of oral fluids (water, ORS, coconut water).
- Take a soft, bland diet. Avoid oily and spicy food for one week.
- Stop TAB TELMA-H for two more days, then restart your regular dose.

FOLLOW-UP INSTRUCTIONS:

- Review in the General Medicine OPD with Dr. R. Mehta after 5 days.
- Please come back to the emergency room immediately if you develop high fever, severe abdominal pain, recurrence of vomiting or decreased urine output.

PENDING RESULTS:

- Urine Culture & Sensitivity Report. Please collect the report during your follow-up visit.